



INPATIENT RECORD

Hospital No: 57018681	Visit No:
Name:	Age/Sex:
Doctor Name:	Specialty:
Date:	

BP Systolic: 120 mmHg	BP Diastolic: 90 mmHg	Pulse Rate: 89 bt/min
Respiration: 16 br/min	Temperature: 98 °F	Delivered Oxygen Flow Rate: 0
Saturation(Oxygen): 98 %	Mean Arterial Pressure-MAP: 100 mmHg	Bowel Status: 1
Eye Opening: 4		

Forms

Reassessment Tool(Pre and Post Shift Out)

Score : 0

Temperature : 98

Pulse : 80

Respiration : 16

SpO2 : 98

BP : 120/90

Shifting : Daycare-Before Discharge

: Time

11:50am-Patient received from the admission desk with a case ADENOCARCINOMA STOMACH. Patient currently occupied day care With complaints of fever and tiredness patient visit informed to Dr Annalakshmi

Discharge Checklist MHE

Status : Authorised

Score : 0

Discharge Planning initiated : 24/03/2026

Reports required by patient on discharge : yes

IV Cannula removed : Yes

Last vital signs documented : Yes

Discharge Medication or Prescription : Yes

Investigation results : Yes

Appointment details : Yes

Advice/Instructions/Education information : Yes

ID band removed : Yes

All personal belongings returned : Not Applicable

Discharge comments : left the ward with stable condition

Reports supplied to patients on discharge : 1 op file in that discharge summary-1 blood reports -4 given to patient husband srinivas

Home Discharge Planning

Intravascular Access Lines

Status : Authorised

IntraVascular Access Type : Peripheral IV Line

Peripheral IV Activity : Discontinued

CVC Site Assesst



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Lumen color : blue
Central IV TPA Intervention

Catheter position corrected / withdrawn

Arterial Line Status

Peripheral IV Assessment

Peripheral IV Removal Date : 24/03/2026

Peripheral IV Removal Time : 18:00

Peripheral Removal Indication : Treatment completed

NURSING INITIAL ASSESSMENT - NEW

Status : Entered

Score : 0

Patient Name : Shobha K

MRN : MH009527399

Age : 51 Yrs

Gender : Female

Patient arrival Date : 24/03/2026

Patient Arrival Time : 11:55

Accompanied by Relative : Yes

Name of Relative : Mr. Shrinivas

Relationship with Patient : Husband

Contact Number : 9845256747

Primary Language Spoken : Kannada

Interpreter Needed : No

Room Orientation Given : Yes

Patient Identification Band Fixed : Yes

If Yes Type Of Band : Green and orange band

Height(Cm) : 161

Weight(Kgs) : 71.7

Reason Not Able To Check : .

Chief Complaints : Currently admitted for immunotherapy#89

Past History : ADENOCARCINOMA STOMACH STAGE IV (PERITONEAL METS)

HYPOTHYROIDISM

FIBROMYALGIA

Vital Assessment

Temp(F) : 98

Pulse/min : 89

Blood Pressure(mmHg) : 120/90

Respiration/min : 16

Spo2(%) : 98

GRBS (mg/dl) : .

Skin Integrity : INTACT

Other Parameters

EWS Score : 0



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GCS : 15/15
Pain Score : 0
Valuable handover checklist completed : Yes
Belongings : With Patient
Dentures : No
Hearing Aid : No
Eye Glasses/Contact Lens : No
Others : .
Vulnerable : Yes
Any Special Care Given : keep side rails up, frequent visiting, no wet floor
Restraints : No
If Yes, Type Of Restraint : NO
Any Known Allergies : unknown
Current Medications : Tab Maxdulin 0-0-1 x continue
Tab Emeset 4mg sos for nausea
Tab Sompraz D 1-0-0 X daily & sos (before food) for gastritis
Syp Mucaine gel 10 ml sos (before food) for mucositis/gastritis
Informed the Doctor : Yes
Name of the Doctor : Dr Annalakshmi S
Investigations Ordered : NIL
Diet : neutropenic diet
Collected Old records : Yes
If Yes Document Type : 1 OP FILE
Is the patient at risk for any of the following? If Yes, complete the risk assessment form.
Fall : YES
Pressure Score : NO
DVT : YES
Aspiration : YES
Pain Assessment Score : Not Applicable
Ability to perform activities of daily living
Activity
Bathing : Independent
Dressing : Independent
Eating : Independent
Walking : Independent
Toilet Use : Independent
Nursing needs(complete Nursing care plan based on the identified needs)
Actual needs : To control the growth of cancer cells
– Palliation.
– make a tumor smaller before surgery/ radiation therapy
– destroy cancer cells that may remain after surgery or radiation therapy
– help other treatments work better
– kill cancer cells that have returned or spread to other parts of your body
Potential needs : Improve quality of life and prolong life expectancy.
-To prevent extravasation
- Managing and relieving of chemo drugs side effects and pain management.
Any other needs : Palliative care or hospice care, which help control the symptoms and side effects of the disease.



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Assessment Done By : Ankitha D.S
Rating : 0
BMI : 27.39
Fall Score : 3-LOW RISK
Pressure score : 23-NO RISK
DVT Score : 4-HIGH RISK
Aspiration score : 0-LOW RISK
General Physical Examination:
Pallor : NO
Icterus : NO
Edema : NO
Clubbing : NO
Cyanosis : NO
Lymphadenopathy : NO
Any Other Findings : .

Forms

Patient/Family Health Education
Status : Authorised
Date : 24/03/2026
Time : 18:29
Assessment Type : Initial Assessment

Educational Needs : Safe and effective use of medical equipment, Discharge Instruction of continuing care, Personal hygiene and practitioner hand wash, Infection control related issues, Home Instruction, Other (specify in comments), Health Maintenance, Social Services, Safety, Hand hygiene, Isolation precaution
Given Education : Maintain body and oral hygiene (Chlorhexidine mouth wash daily)
2. Avoid contact with sick people (fever, cough, running nose)
3. Avoid going in crowded places.
4. Avoid exposure to dust
Teaching Method : One to One, Group teaching
Teaching Tool : Verbal Instructions
Evaluation : Applies knowledge/skills
Child Attends School : No
Does the child has any problem at school : No
Literacy Skills : No
Plan and Comments : Report back if abdominal pain, abdominal distension, fever, loose stools > 3 times per day, bleeding manifestations, excessive tiredness, breathing difficulty, altered sensorium or SOS in case of any undue symptoms.
Add Nursing Care Plan
Status : Entered
Nursing Care Plan discussed with Patient : Yes
Nursing Care Plan discussed with Family : Yes
Patient Measurable Goal for the day : To reduce the risk of DVT and their complication
Nursing Diagnosis : Risk for DVT
Education and comments : Taking anticoagulants (blood thinners) as prescribed for the appropriate duration, depending on the risk factors and the location of the clot.
- Monitor for excessive bleeding and adjusting the diet if taking warfarin, which can interact with Vit K rich foods.



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- Diagnosing DVT using the wells score, D -dimer test and venous ultrasound.
- Avoid any procedures including tooth extraction strictly; to inform prior to any procedures
- Avoid intramuscular injections;
- Avoid NSAID's (Paracetamol is safe)
- No self-medications strictly
- To watch out for any bleeding manifestations and report immediately
- To withhold apixaban if plt count <50,000 and inform

Nursing Interventions : Maintain body and oral hygiene (Chlorhexidine mouth wash daily)

2.Avoid contact with sick people (fever ,cough, running nose)

3.Avoid going in crowded places.

4.Avoid exposure to dust

Evaluation : Continue

Progress Note :

Seen by Dr.Satish Kumar A

51 yr old lady

ADENOCARCINOMA STOMACH STAGE IV (PERITONEAL METS)

HYPOTHYROIDISM

FIBROMYALGIA

Dec 23; Venous Doppler of Left lower limb ;

Great saphenous vein at the upper 1/3 of the leg and its tributaries are dilated and shows heteroechoic thrombus with no flow – thrombophlebitis.

On Tab Apixaban (Eliquis) 2.5mg 1-0-1 (8am and 8pm)

On Nivolumab Q3weeks (last on 2 March 26)

Referred in view of thrombocytopenia

No bleeding manifestations

Had UTI recently; On NIFTAS

2/3/2026;Hb-11.3;MCV-82.3;Tc-8270;N66.8;L24.8;plt c-153000

9/3/2026;Hb-11;MCV-87.3;Tc-7620;N51.4;L40.8;plt c-121000

Urine c/s - Escherichia coli 10³ cfu/ml

12/3/2026;Hb-10.9;MCV-86.6;Tc-6880;N55.6;L36.9;plt c-75,000

17/3/2026;Hb-10;MCV-88.2;Tc-3860;N46.1;L43.8;plt c-60000

Manual platelet count done, few giant platelets noted

24/3/2026;Has come for Follow up

Received Inj Romiplostim 250mcg on 17/3/2026

Asymptomatic

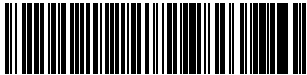
no bleeding manifestation

Inv;cr-1.08

Hb-10.9;MCV-86.5;Tc-4870;N42.5;L46.4;plt c-150000

PS-Normocytic Normochromic Anemia.

LDH-238;tb-0.57;Alb-3.9;Glo-2.1;A/G-1.9;sgpt-25



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Imp ; Thrombocytopenia - probably acute onset

Adv;
Inj Romiplostim 250mcg s/c stat
- Follow up after 2 WEEKs with cbc

MEDICATION ORDERS				
Drug / Generic Item	Dosage Qty	Frequency	Instructions/Notes	Duration
MEDISET 4mg / 2 ML AMP INJ (ONDANSETRON (80007494))	2 AMP	ONCE		1 DAY(S)
PANTOCID 40 MG/VIAL LYOPHILIZED POWDER FOR INJECTION (PANTOPRAZOLE (80007606))	1 VL	ONCE		1 DAY(S)
DOLO 650 MG TABLET (PARACETAMOL (80007661))	1 TAB	ONCE		1 DAY(S)
AVIL 45.50 MG/2 ML AMPOULE SOLUTION FOR INJECTION, 22.75 MG/ML (PHENIRAMINE (80007868))	1 VL	ONCE		1 DAY(S)
OPDYTA INJECTION 100MG (NIVOLUMAB)	1 POWDERFORINJECTION	ONCE		1 DAY(S)
OPDYTA 40 MG INJECTION (NIVOLUMAB)	2 POWDERFORINJECTION	ONCE		1 DAY(S)
NORMAL SALINE 09% W/V AQUA PULSE 500 ML (SODIUM CHLORIDE (80009680))	1 BT	ONCE		1 DAY(S)
NORMAL SALINE 0.9% W/V 100 ML AQUA PULSE, DENIS (SODIUM CHLORIDE (80009680))	3 BT	ONCE		1 DAY(S)
AUTOFUSION SET # 14289 (IV SET (PHARMACY NON-DRUG))	1 PC	ONCE		1 DAY(S)
TEGADERM VELFIX EDGE 7CMX9CM #5502A,DATT	1 PC	ONCE		1 DAY(S)
22G VENFLON(OHMEDA) (CANNULA (PHARMACY NON-DRUG))	1 PC	ONCE		1 DAY(S)
POLYFLUSH PRE-FILLED SYRINGE 5 ML # 90319, POLY MEDICURE LTD (SYRINGE (PHARMACY NON-DRUG))	5 PC	ONCE		1 DAY(S)
SYRINGE 10 ML WITH NEEDLE DISPOVAN, HMD (SYRINGE (PHARMACY NON-DRUG))	4 PC	ONCE		1 DAY(S)
SYRINGE 2 ML WITH NEEDLE DISPOVAN, HMD) (SYRINGE (PHARMACY NON-DRUG))	3 PC	ONCE		1 DAY(S)
ROMY 250 MCG/0.5 ML VIAL INJECTION (ROMIPLOSTIM 250 MCG INJECTION)	1 VL	ONCE		1 DAY(S)
NEUROBION FORTE-RF (1000mcg, 100mg, 100mg) solution for injections [2ml] (THIOCTIC ACID~BENFOTIAMINE~CHROMIUM~FOLIC ACID~INOSITOL~VITAMIN B12~PYRIDOXINE)	1 SOLUTION FOR INJECTIONS	ONCE		1 DAY(S)